A/PROF CHAMARA BASNAYAKE

MBBS, PhD, FRACP

Gastroenterologist & Endoscopist

PN 421926PK

FOCUS GASTROENTEROLOGY

29 April 2026

Dr Jamie Patel
Collingwood Medical

Dear Dr Patel,

Re: Mr Brett Willemsen
DOB: 25/03/1990 Phone: 0493 206 910
Address: 2/99 Alexandra Avenue, SOUTH YARRA, VIC, 3141
Email: bzmwillemsen@gmail.com
Medicare: 3591736261 / 1

Summary

- **Gastroscopy and colonoscopy excluding coeliac disease, Helicobacter pylori, inflammatory bowel disease, and microscopic colitis.**
- **Excision of a colonic polyp (Tubulovillous adenoma) requiring surveillance in three years.**
- **Clinical diagnosis of visceral hypersensitivity secondary to stress and SSRI cessation.**
- **Commencement of conservative management including fibre supplementation and dietary modifications.**
- **Action suggested Please consider a Mental Health Care Plan to facilitate gut-directed hypnotherapy.**

I had the pleasure of reviewing Brett following his recent gastroscopy and colonoscopy. These investigations were performed to investigate a change in bowel habit and increased belching, which notably commenced following the abrupt cessation of sertraline during a period of significant psychological stress.

I am pleased to report that the gastroscopy was macroscopically and histologically normal. Biopsies of the small intestine confirmed no evidence of coeliac disease, and gastric biopsies were negative for *Helicobacter pylori* and autoimmune gastritis. The oesophagus also appeared healthy. Similarly, the colonoscopy excluded macroscopic inflammatory bowel disease. Random biopsies were taken which ruled out microscopic colitis and histological evidence of Crohn's disease or ulcerative colitis.

A polyp was identified and successfully excised . While I initially suggested a five-year surveillance interval, upon further review of the histology (TVA), I have recommended a follow-up colonoscopy in three years to mitigate future risk.

Impression and Plan

Brett's symptoms are highly suggestive of visceral hypersensitivity. It appears that a combination of autonomic nervous system arousal and the removal of the neuromodulatory effect of his SSRI has led to heightened sensory perception within the gut. He describes classic features of post-prandial urgency and loose stools, particularly triggered by high-fat meals.

All appointments and enquires

Suite 201, 100 Victoria Parade
East Melbourne VIC 3002
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Pager: 03 9387 1000
Email: office@focusgastro.com.au

Endoscopy locations

Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

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We have opted for a conservative management strategy. I have advised Brett to cease the daily use of chewing gum, as the artificial sweeteners (such as sorbitol) likely contribute to his symptoms of bloating and diarrhoea. To improve stool consistency, I have recommended he begin a fibre supplement, specifically partially hydrolysed guar gum (PHGG), starting at one teaspoon daily and titrating up to four as tolerated.

Brett is also keen to pursue gut-directed hypnotherapy, which has a robust evidence base for managing these symptoms. I have referred him to The Gut Centre. If you feel it is appropriate, providing him with a Mental Health Care Plan would assist in facilitating these sessions from a financial POV. While we discussed the potential use of low-dose nortriptyline to address his symptoms, Brett prefers to exhaust non-pharmacological options first. I will review him on an as-needed basis should his symptoms fail to improve with these measures. Thank you very much for this referral

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

Investigation Results:

Willemsen, Brett : EPW 3119238

DOB: 25/03/1990 Age: 36 years

2/99 Alexandra Avenue, South Yarra 3141

Results from 14/04/2026 ..... Episode# 26P276031 Test Hist (Histology - Routine)

FINAL ANATOMICAL PATHOLOGY AAnat

Lab #: 2026B009593 **CLINICAL NOTES** Gas N, Duo N, random colon N, ascending polyp= TA LLD, TI= mild ileitis. **MACROSCOPIC DESCRIPTION** 1. Specimen labelled "Gastric". Two mucosal fragments measuring 2-4 mm. A1. 2. Specimen labelled "Duodenum". Three mucosal fragments measuring 2-3 mm. A1. 3. Specimen labelled "TI". Two mucosal fragments measuring 3-4 mm. A1. 4. Specimen labelled "Ascending Colon polyp". Two tan polypoid fragments 2 x 1 x 1 mm and 3 x 1 x 1 mm. Processed whole. A1. 5. Specimen labelled "Random Colon". Three mucosal fragments measuring 2-6 mm. A1. **MICROSCOPIC DESCRIPTION And DIAGNOSIS** 1. Gastric - Gastric mucosal tissue with no significant pathological features. There is no evidence of intestinal metaplasia or gland atrophy. No Helicobacter pylori-like organisms are seen. No evidence of dysplasia or malignancy. 2. Duodenum - Duodenal mucosal biopsies with no significant pathological features. Morphological features of coeliac disease are not seen. No granulomas or parasites are identified. No evidence of dysplasia or malignancy. 3. TI - Terminal ileum mucosal biopsies with no significant pathological features. No parasites or granulomas are identified. No evidence of dysplasia or malignancy. 4. Ascending colon polyp - Tubulovillous adenoma with mild and moderate dysplasia (low grade). Severe dysplasia (high grade) or malignancy is not seen. 5. Random colon - Large bowel mucosal biopsies with no significant pathological features. There is no ulceration, architectural distortion or "bottom heavy"

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inflammation. No granulomas are seen. There is no evidence of microscopic colitis. No parasites are seen. There is no evidence of dysplasia or malignancy. Pathologist: Dr Cesar Salinas-LaRosa

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